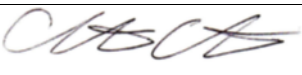


Guideline Title: General Patient Care

Guideline Number: GUID-3203-G002

Issue date: 09/09/2014	Replaces Dept. Policy:
Revision dates: 4/30/24	Developed by: Air Care Team
Approved by: Dr. Christopher Hunter, MD Air Care Team Medical Director	Approved by:
Signature: 	Signature:
Department Numbers: 3203	

- I. **PURPOSE:** Regardless of the nature of the illness or injury, the following minimum patient care guidelines will be provided to all patients that are treated and transported by the Air Care Team.
- II. **DEPARTMENT GUIDELINE:**
 - a. All AIR CARE TEAM aircraft will be staffed with a minimum of one Registered Nurse and one licensed Paramedic.
 - b. Standard precautions and personal protective equipment (PPE) will be utilized for all patient care. Medical crew will wear additional PPE according to other organizational or department standards if patient is on contact, droplet, or airborne precautions.
 - c. Consent for patient transport must be obtained by the patient and the Authorization and Consent Form signed unless the patient is mentally or physically unable to sign.
 - i. Acceptable reasons for the patient not to sign include unconscious or unresponsive, cardiac/respiratory arrest, altered mental status, intellectual disability, physical disability that prohibits signing, a minor, a prisoner/inmate in custody, has a legal guardian appointed (not power of attorney unless meets criteria), or involuntary psychiatric transfer.
 - d. An initial assessment will be performed on all patients to identify abnormalities and/or injuries.
 - e. All patients will be connected to the cardiac monitor, pulse oximeter, and blood pressure monitor as soon as practical and remain on these monitoring devices until care is turned over at the receiving care facility.
 - f. The following will be monitored and recorded: heart rate, blood pressure (non-invasive or invasive), pulse oximetry, end-tidal carbon dioxide (ETCO₂, if indicated), and respiratory rate a minimum of every 10 minutes for inter-facility flights and every 5 minutes for scene flights. The frequency of monitoring vital signs will be advanced as determined by patient's condition.
 - g. Supplemental oxygen will be placed on the patient as appropriate to their condition prior to flight for those patients not currently on oxygen to maintain oxygen saturations > 92%.
 - h. Patients administered parenteral narcotics or sedative agents must have continuous cardiac and SpO₂ monitoring. End Tidal CO₂ monitoring should be used all non-intubated patients with a GCS < 14 who receive sedation or analgesia.

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- i. All patients shall have a patent airway prior to transport. Patients who are unable to protect their own airway should have their airway secured.
 - i. ECG, pulse oximetry, ETCO₂ monitoring is mandatory for all advanced airway procedures to include intubation, supraglottic airway, or cricothyrotomy.
 1. Intubated patients will have their tube placement confirmed prior to departure from the scene or facility by confirmation of ETCO₂ detection, either qualitative or quantitative.
 2. Continuous ETCO₂ monitoring is mandatory on all intubated patients.
 3. Intubated patients ideally should be ventilated utilizing the transport ventilator. In some circumstances, such as very short flight time or patient inability to tolerate mechanical ventilation, ventilations via bag-valve would be an acceptable alternative.
 4. Intubated patients should have a gastric tube placed if not contraindicated and if time permits.
- j. Establish NIBP monitoring except for patients with invasive monitoring. Establish invasive pressure monitoring on all patients with arterial lines.
- k. Intravenous (IV) or intraosseous (IO) access is to be considered mandatory unless the attempts at access delay essential care or if it is inappropriate for the patient's condition (e.g., epiglottitis). If unable to gain IV access, consider IO access for those critically ill patients where access is mandatory.
 - i. All IV/IO devices will be checked for patency prior to departure and throughout transport from the scene or sending facility.
- l. Trauma patients and patients with potential for hypotension ideally will have two large bore IV lines.
- m. All IV medications hung prior to and during transport will be properly labeled with the time the drug was started, the name of the drug, the dosage, and the concentration of the solution.
- n. All IV infusions will be regulated with the use of an IV infusion pump. The only exceptions will be the use of maintenance IV fluids without additives (for adults only) and the use of an IV for rapid fluid resuscitation and blood replacement.
- o. Invasive lines will be transduced and monitored as appropriate.
- p. All pediatric patients will have their weights verified by using a length-based resuscitation tape.
- q. All patients with altered mental status will have a blood glucose evaluation unless a blood glucose level from the referring facility is normal.
- r. All pediatric patients will have a blood glucose done unless a blood glucose level from the referring facility is normal.
- s. All patients will be assessed for pain.
- t. All pediatric and neonatal patients will have temperature documentation.
- u. For trauma situations, a pediatric patient is defined as a person fifteen (15) years or age or younger.
- v. All patients will be monitored for comfort and warmth.

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- w. All medication administration will be performed in compliance with Orlando Health Patient Care Policy and Procedures, including double verification of high-risk substances and observation of the “5 R’s” (right drug, right patient, right route, right time, right documentation)
- x. All patients will be reassessed after each intervention and medication administration and their response will be documented.
- y. All patients will be secured to the aircraft stretcher with 3 straps and shoulder harness and if necessary, will be chemically or physically restrained for safety of patient and flight team. The neonatal transport team will secure the infant in their isolette.
- z. All patients will be provided hearing protection.
- aa. All equipment shall be secured before takeoff.
- bb. The team will ensure patient/family receives information regarding transport.
- cc. The team will ensure patient privacy is respected.
- dd. The team will ensure patient information is kept confidential.
- ee. The team is responsible for transporting complete patient records including original or copies of patient radiology scans with the patient as received by sending nurse/institution. If results are pending, the medical transport team should request that they be faxed to the receiving facility.
- ff. If transferring a pediatric patient to the pediatric intensive care unit, an updated report should be called to the receiving physician prior to departure from the unit of the sending facility.
- gg. The team will contact the emergency department (ED) via radio with a patient report and estimated time of arrival for all patient’s arriving from a scene call or for interfacility transfers to the ED at least 5 minutes prior to arrival.
- hh. Orders provided by the physician should be followed unless, in the judgment of the team, they endanger the patient. If a receiving physician gives orders to perform a procedure that is not covered in the Air Care Medical Treatment Protocols but is within the scope of practice of a Registered Nurse or Paramedic and the individual has been trained to perform said procedure, perform the procedure in accordance with standards set for the level of certification. To request any other orders not covered in the Air Care Medical Treatment Protocols, or clarify any clinical concerns call Medical Control (Dr. Christopher Hunter, 914-552-8954).
- ii. The team will provide complete a complete report and provide a completed AIR CARE TEAM HANDOFF SHEET to the receiving nurse.
- jj. A patient care record will be completed following each patient flight in the current department electronic health record.

III. **DOCUMENTATION:**

IV. **REFERENCES:** None